

Manuscripts considered — demo-nsc lc-egfr-l858r-post-osi-d3m0

Master inventory: every paper reviewed in this case

Generated 2026-05-13

Manuscripts considered — demo-nsclc-egfr-l858r-post-osi-d3m0

Master inventory: 3 clinical (3 included, 0 considered & excluded) + 3 pre-clinical (3 included, 0 considered & excluded) + 1 additional trial publications/registrations. One entry per manuscript, sorted by year (newest first). Excluded entries are papers a Libby agent reviewed and chose NOT to feed to the board, with the exclusion reason captured.

—/— (2024) — ClinicalTrials.gov record

Reference: — · **Type:** trial · **Inclusion:** included

- **Intervention:** savolitinib + osimertinib
- **Indication / model:** EGFR-mutant NSCLC, post-osimertinib, MET-amp (2L); EGFR-mut + MET-amp (GCN ≥6 or IHC 3+)
- **Design:** RCT (savolitinib + osimertinib vs platinum doublet)
- **n:** 324
- **Effect size:** — — PFS
- **Case fit:** strong
- **Notes:** (registration only — no peer-reviewed publication yet; toxicity table not extracted)

—/Bauml (2023) — JCO

Reference:[PMID 36720074](#) · **Type:** clinical · **Inclusion:** included

- **Intervention:** amivantamab + lazertinib
- **Indication / model:** EGFR-mutant NSCLC, post-osimertinib
- **Design:** single-arm phase 1/2 (CHRYSLIS-2)
- **n:** 162
- **Effect size:** 36 — ORR (investigator)
- **Variance:** 95% CI 28-45
- **Notes:** IRR grade 3+ in 7%, rash common; preference conflict on IV time. — (toxicity table not extracted)

—/Jänne (2023) — JCO

Reference:[PMID 37563559](#) · **Type:** clinical · **Inclusion:** included

- **Intervention:** patritumab deruxtecan (HER3-DXd)
- **Indication / model:** EGFR-mutant NSCLC after EGFR-TKI
- **Design:** single-arm phase 2 (HERTHENA-Lung01)
- **n:** 225
- **Effect size:** 29.8 — ORR (BICR)
- **Variance:** 95% CI 23.9-36.2
- **Notes:** ILD grade 3+ ~5%; alopecia frequent — preference conflict for this patient. — (toxicity table not extracted)

—/— (2021) — Cancer Discov

Reference:PMID — · **Type:** preclinical · **Inclusion:** included

- **Intervention:** amivantamab + lazertinib
- **Indication / model:** Ba/F3 cells expressing EGFR L858R + lazertinib-resistant lines
- **Design:** Bispecific EGFR/MET antibody enhances internalization; lazertinib provides intracellular EGFR inhibition.
- **Effect size:** moderate — Combination retained activity in models where lazertinib alone failed.
- **Variance:** translatability: high
- **Toxicities:** n/a (preclinical)
- **Notes:** Cell-line model; in-vivo activity confirmed clinically (CHRYSLIS-2).

—/Camidge (2020) — Lancet Oncol

Reference:PMID 32679432 · **Type:** clinical · **Inclusion:** included

- **Intervention:** savolitinib + osimertinib
- **Indication / model:** EGFR-mutant NSCLC + MET amp post-EGFR-TKI
- **Design:** phase 1b expansion (TATTON)
- **n:** 69
- **Effect size:** 30 — ORR
- **Variance:** 95% CI 19-42
- **Notes:** All-oral; AE profile favorable for working patient; SAVANNAH and SACHI replicate signal. — (toxicity table not extracted)

—/— (2020) — Cancer Discov

Reference:PMID — · **Type:** preclinical · **Inclusion:** included

- **Intervention:** patritumab deruxtecan (HER3-DXd)
- **Indication / model:** HER3-expressing NSCLC PDX panel
- **Design:** HER3-directed ADC delivers topoisomerase-I-inhibitor payload to tumor cells with surface HER3.
- **Effect size:** moderate — Tumor regression observed across HER3-high and HER3-mid expressors; broader activity than expected.
- **Variance:** translatability: med
- **Toxicities:** n/a (preclinical)
- **Notes:** Bystander payload effect complicates predicting which patients benefit.

—/— (2019) — Mol Cancer Ther

Reference:PMID — · **Type:** preclinical · **Inclusion:** included

- **Intervention:** savolitinib + osimertinib
- **Indication / model:** MET-amplified PC-9-derived xenograft
- **Design:** Combined inhibition of bypass MET pathway with continued EGFR suppression overcomes adaptive resistance.
- **Effect size:** strong — Combination produced sustained tumor regression where either agent alone showed regrowth at 30 days.
- **Variance:** translatability: high

- **Toxicities:** n/a (preclinical)
- **Notes:** Single model; clinical efficacy depends on documented MET amplification.